

## TRIDEX SYRUP

An orange color syrup with tutti-frutti flavour.

### **PHARMACOLOGY:**

Tripolidine, a histamine H<sub>1</sub>-receptor antagonists, competitively inhibit most of the pharmacologic actions of histamine. It effectively antagonizes the action of histamine that results in increased capillary permeability and the formation of edema. This H<sub>1</sub>-receptor antagonist also suppresses flare and pruritis which accompany the endogenous release of histamine.

Dextromethorphan retains only the antitussive activity of other morphinan derivatives. It is about equal to codeine in depressing the cough reflex and has no expectorant action. In therapeutic dosage, dextromethorphan does not inhibit ciliary activity.

Pseudoephedrine acts directly on the alpha-adrenergic receptors in the mucosa of the respiratory tract producing vasoconstriction which results in shrinkage of swollen nasal mucous membranes, reduction of tissue hyperemia, edema and nasal congestion, and an increase in nasal airway patency.

### **PHARMACOKINETICS:**

Tripolidine is readily absorbed from the gastrointestinal tract after oral administration. It is mostly metabolised in the liver and excreted mainly as inactive metabolites in the urine. It has a rapid onset and long duration of action. Following oral administration of antihistamines, symptomatic relief of allergic reactions usually begins within 15-30 minutes and usually the maximum effect occurs within 1 hour.

Dextromethorphan is rapidly absorbed from the GI tract and exerts its effect in 15-30 minutes after oral administration. The duration of action is approximately 3 to 6 hours with conventional dosage forms.

Pseudoephedrine is rapidly absorbed from the GI tract and exerts its effect within 30 minutes after oral administration of 60mg of pseudoephedrine hydrochloride and persists for 4-6 hours. Pseudoephedrine is incompletely metabolized in the liver to an inactive metabolite. 55-75% of the drug is excreted unchanged in urine, together with its metabolites. The rate of urinary excretion of pseudoephedrine is accelerated when urine is acidified to a pH of about 5 and slowed when urine is alkalinized to a pH of about 8.

### **COMPOSITION:**

Each 5ml syrup contains Tripolidine Hydrochloride 1.25mg, Dextromethorphan Hydrobromide 15mg and Pseudoephedrine Hydrochloride 30mg.

Sodium benzoate 0.2% w/v as preservative.

Alcohol 95% ( ethanol 95% ) 5% v/v

### **INDICATIONS:**

It is indicated to relieve dry irritating coughs, which are often associated with common cold, upper respiratory tract infections, allergic rhinitis (seasonal and perennial) and hay fever. Helps dry up running nose and opens congested bronchi to aid clear breathing.

### **DOSAGE/ADMINISTRATION:**

To be taken orally. Preferred to be sipped and swallowed slowly, without dilution.

#### **Dosage for adults and children above 12 years:**

10ml syrup three times daily.

#### **Children 6 to 12 years:**

5ml syrup three times daily

#### **Children 2 to 5 years;**

2.5ml syrup three times daily

**ADVERSE EFFECTS:**

Tridex Syrup may cause drowsiness, dizziness and constipation. Other side effects that may occur include gastrointestinal discomfort.

Other less common side-effects may include transient hypertension, dry mouth, insomnia, restlessness, palpitations, allergic reactions such as rashes, tightness of chest, thickening of bronchial secretions, toxic psychosis and blood dyscrasia.

**INTERACTIONS WITH OTHER MEDICAMENTS:**

1. Additive CNS depression may occur when antihistamines are administered concomitantly with other CNS depressants including barbiturates, tranquilizers and alcohol. Patients already receiving another CNS depressant (e.g., sedatives, tranquilizers) should not undertake self-medication with an antihistamine without first consulting a physician.
2. Monoamine oxidase (MAO) inhibitors, by increasing the quantity of norepinephrine in adrenergic nervous tissue, potentiate the pressor effects of pseudoephedrine. MAO will also prolong and intensify the anticholinergic effects of antihistamines.
3. Possible interactions may occur with betahistine, certain hypnotic and anxiolytic medication, metoclopramide and antiparkinson drugs.

**PRECAUTIONS/WARNINGS:**

Use with caution in patients with epilepsy, prostatic hypertrophy, glaucoma, hepatic disease, hypertension, heart disease, diabetes, hyperthyroidism, patients with stenosing peptic ulcer, pyloro-duodenal obstruction or bladder neck obstruction, unless under the medical advice and supervision.

This syrup contains pseudoephedrine and dextromethorphan which should not be used in patients receiving monoamine oxidase inhibitors.

This product may cause drowsiness and dizziness. As such, patients should be warned to avoid activities that require mental alertness such as driving or operating machineries. Patients should also abstain from alcohol.

If symptoms do not improve within one week or accompanied with high fever, please consult a physician's advice before continuing use.

When used for treatment of cough and cold:

- i) Not to be used in children less than 2 years of age.
- ii) To be used with caution and on doctor's or pharmacist's advice in children 2 to 6 years of age.

**PREGNANCY AND LACTATION:**

Safe use of antihistamines and pseudoephedrine during pregnancy has not been established; therefore, this syrup should not be used in women who are or may become pregnant unless the potential benefits justify the possible risks to the fetus.

This drug may be distributed into milk and should not be given to nursing women because infants may be especially susceptible to its effects.

**CONTRAINDICATIONS:**

Hypersensitivity to triprolidine hydrochloride, pseudoephedrine hydrochloride or dextromethorphan hydrobromide or other similar chemical structure. It should be avoided in patients with liver disease or asthmatic patients and is contraindicated in patients taking monoamine oxidase inhibitors or within 2 weeks from stopping such treatment and during acute attacks of asthma. Not suitable for patients with severe hypertension or severe coronary artery disease, and those who are receiving monoamine oxidase inhibitors.

Tridex syrup should not be taken for persistent or chronic cough (eg with smoking, emphysema, asthma) or when coughing is accompanied by excessive secretions unless directed by a physician.

**SYMPTOMS AND TREATMENT FOR OVERDOSAGE:**

Symptoms of overdosage include respiratory depression, drowsiness, hypotension, paranoid psychosis, delusions, hallucinations, palpitation, restlessness and/or convulsion.

Treatment: In general, the management of overdosage involves supportive and symptomatic therapy; maintaining and supporting respiration and circulation should be prioritized. Treatment should include emptying the stomach by

aspiration or gastric lavage. Nervous stimulation and convulsions should be treated with a sedative such as diazepam intramuscularly. If marked excitation present, a sedative such as diazepam or a short acting barbiturate may be given. Elimination of pseudoephedrine may be assisted by acidification of the urine with ammonium chloride.

**PHARMACEUTICAL INFORMATION / PACKAGE:**

**Description** : An orange color syrup with tutti-frutti flavour.  
**Packing Size** : ~90 ml (amber plastic bottle)  
**Storage** : Store in the original package and in a dry place. Do not store above 30°C. Protect from light.  
**Shelf Life** : 3 years from date of manufacturing.

**MANUFACTURER & PRODUCT REGISTRATION HOLDER:**

MALAYSIAN PHARMACEUTICAL INDUSTRIES SDN BHD (101323-U)  
Plot 14, Lebuhraya Kampung Jawa, 11900 Bayan Lepas,  
Pulau Pinang, Malaysia.

**DATE OF INFORMATION**

19/10/2017